

Restraint and Seclusion

Policy Number	POL-NUR-005	Owner / Department	Nursing / Patient Care Services
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Last Revised	2026-02-15	Next Review	2028-02-15
Approved By	Patient Safety & Quality Committee	Applies To	All inpatient units, the ED, and behavioral-health areas

1. Purpose

To ensure restraint and seclusion are used only when less restrictive interventions have failed, for the minimum necessary time, consistent with patient rights under CMS Conditions of Participation (42 CFR 482.13).

2. Definitions

- **Restraint** — any manual method, physical or mechanical device, or drug used to restrict movement that is not a standard treatment for the patient's condition.
- **Seclusion** — involuntary confinement of a patient alone in a room from which they are prevented from leaving.
- **Violent/self-destructive behavior** — behavior that jeopardizes the immediate physical safety of the patient or others.

3. Policy — Orders

IMPORTANT — Restraint or seclusion requires an order from a physician or licensed practitioner responsible for the patient's care. PRN (as-needed) and standing orders are prohibited. For violent/self-destructive behavior, the patient must be seen face-to-face within 1 hour of initiation.

- The 1-hour face-to-face evaluation may be performed by a physician/LP or by a trained registered nurse or physician assistant, who then consults the attending.
- Orders for violent/self-destructive behavior are time-limited and may be renewed within the maximum durations below, up to a total of 24 hours; after 24 hours a new in-person evaluation is required before any further order.

4. Maximum Order Durations (Violent / Self-Destructive Behavior)

Patient age	Maximum per order
Adults (18 and older)	4 hours
Children / adolescents ages 9–17	2 hours
Children under age 9	1 hour

Restraint used for non-violent, non-self-destructive reasons (e.g., to protect a medical device) follows separate order and renewal rules and is reassessed at least every 24 hours by the provider.

5. Monitoring and Documentation

- Continuous monitoring appropriate to the type of restraint/seclusion and the patient's condition; assessment of circulation, nutrition, hydration, toileting, and comfort.
- Document the less-restrictive measures attempted, the order, the 1-hour face-to-face findings, monitoring, and the clinical justification for continuation.
- Any death that occurs while a patient is in restraint or seclusion, or within 24 hours after removal, is a SEV-1 event and is reported to CMS as required.

6. References

- CMS Conditions of Participation, 42 CFR 482.13(e)–(g) — Patients' Rights.
- The Joint Commission, Provision of Care standards on restraint and seclusion.
- Related: POL-RM-013 (severity & reporting).